Apollo Hospital Operations & Clinical Performance: FY Analytical Review

Strategic Insights on Revenue Drivers, Departmental Efficiency, and Patient Outcomes

PORTFOLIO DIAGNOSTIC BASELINE | DATA SAMPLE: 5,000 PATIENT RECORDS

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Executive Synthesis & Macro Performance

₹216.54 Cr	6.8 Days	17%	4.03 / 5
TOTAL REVENUE	AVERAGE LOS	OVERALL READMISSION	AVERAGE SATISFACTION

01 Operational Volume

The facility processed 5,000 patients, generating ₹216.54 Cr in total treatment revenue with a baseline Length of Stay (LOS) of 6.8 days.

02 Cost/Quality Divide

Orthopedics delivers optimal efficiency (**4.51/5** satisfaction at moderate cost), while Oncology demands extreme resource intensity (**₹94k** avg cost, **11.6** day LOS).

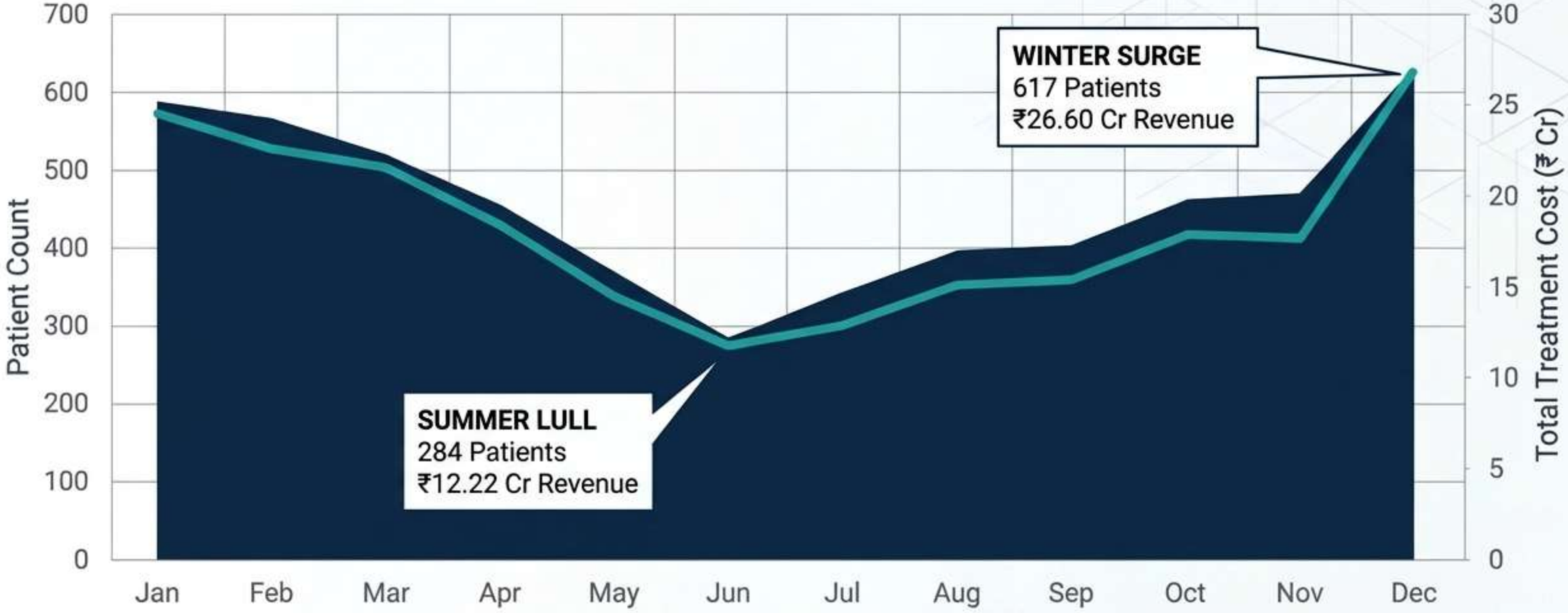
03 Clinical Risk

The baseline **17%** readmission rate masks severe clinical bottlenecks, notably a massive **33%** readmission spike linked to premature discharges in Cardiology.

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Revenue and Volume Seasonality

Patient intake and corresponding revenue peak sharply in Q4 (Winter Surge) and hit their nadir in Q2 (Summer Lull), requiring dynamic resource scaling.



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Revenue Architecture by Payer Profile

Private insurance profiles maximize yield, driving the highest cost per day and average cost. Government schemes (Ayushman) ensure steady volume but yield significantly lower margins.

PRIVATE

Volume: 1,231 Pts

Daily Yield:

₹7,809 / Day

Avg Cost: **₹54,504**

Total Rev: **₹67.09 Cr**

CASH

Volume: 1,465 Pts

Daily Yield:

₹7,097 / Day

Avg Cost: **₹45,581**

Total Rev: **₹66.77 Cr**

CGHS

Volume: 1,004 Pts

Daily Yield:

₹5,542 / Day

Avg Cost: **₹40,019**

Total Rev: **₹40.17 Cr**

AYUSHMAN

Volume: 1,300 Pts

Daily Yield:

₹4,938 / Day

Avg Cost: **₹32,684**

Total Rev: **₹42.48 Cr**

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Departmental Diagnostic Matrix

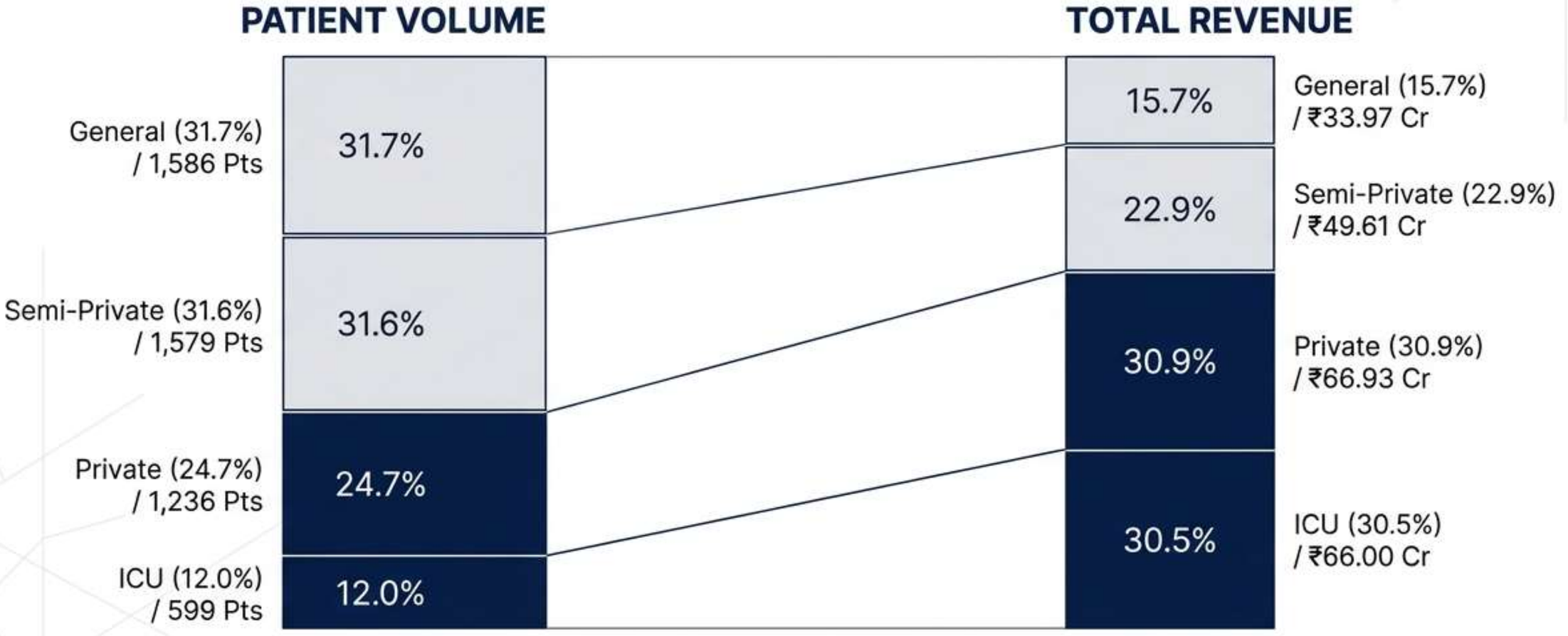
Resource intensity and clinical quality vary drastically by specialty, isolating Orthopedics as the volume/quality leader and Oncology as the high-cost/high-risk outlier.

DEPARTMENT	PATIENTS	AVG LOS	AVG COST	TOTAL COST	SATISFACTION	READMIT RATE
Cardiology	715	7.7	₹48,753	₹34.86 Cr	4.01	26%
Gastroenterology	728	5.3	₹31,838	₹23.18 Cr	3.95	12%
General Medicine	1008	3.8	₹22,434	₹22.61 Cr	4.11	13%
Neurology	493	9.1	₹62,194	₹30.66 Cr	3.79	10%
Oncology	517	**11.6**	**₹94,728**	₹48.97 Cr	**3.60**	**28%**
Orthopedics	**1034**	6.9	₹37,927	₹39.22 Cr	**4.51**	14%
Pulmonology	505	6.0	₹33,737	₹17.04 Cr	3.69	20%
Grand Total	**5000**	**6.8**	**₹43,308**	**₹216.54 Cr**	**4.03**	**17%**

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The Resource Allocation Divide

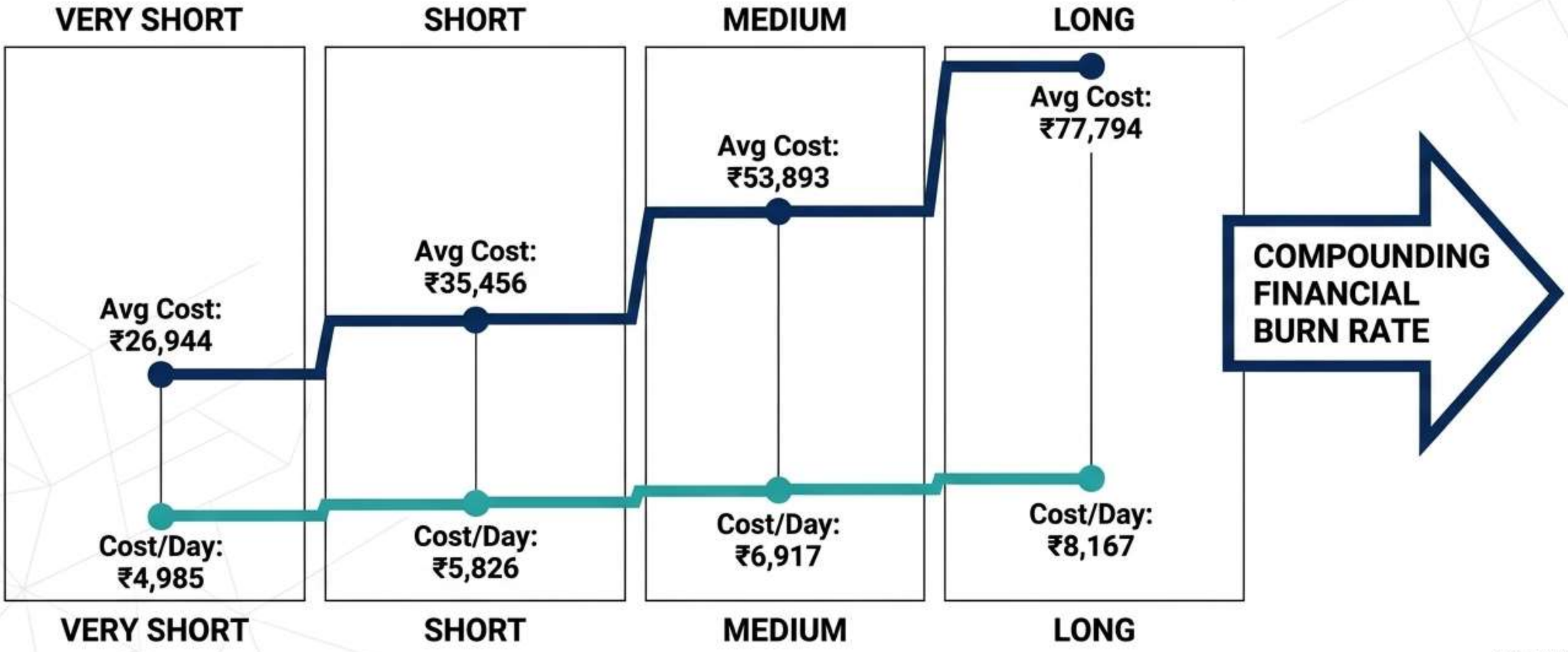
General and Semi-Private beds house 63% of the patient population (3,165 patients), but Private and ICU beds generate the lion's share of aggregate revenue (₹132.94 Cr combined).



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Explanatory Model: The Cost-Duration Curve

Prolonged hospitalizations compound financial intensity. As Length of Stay scales from “Very Short” to “Long,” the average base cost triples, and the operational cost per day escalates by 63%.



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Clinical Quality vs. Cost Effectiveness

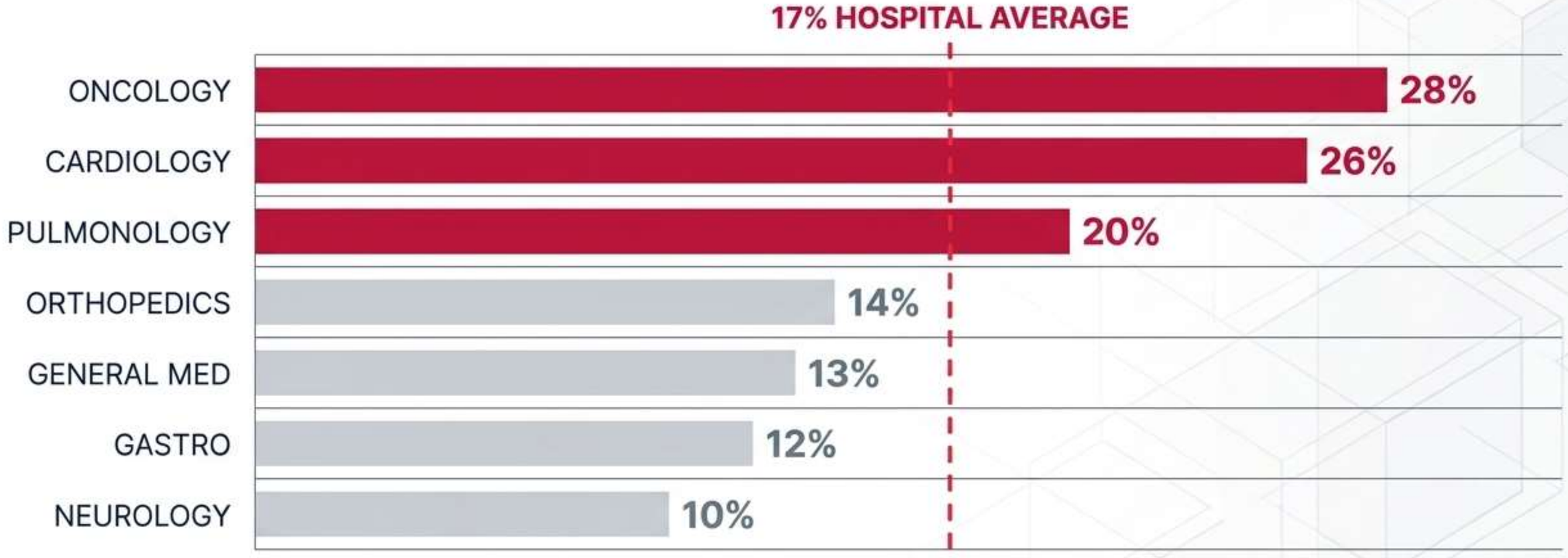
High resource deployment does not guarantee high patient satisfaction.



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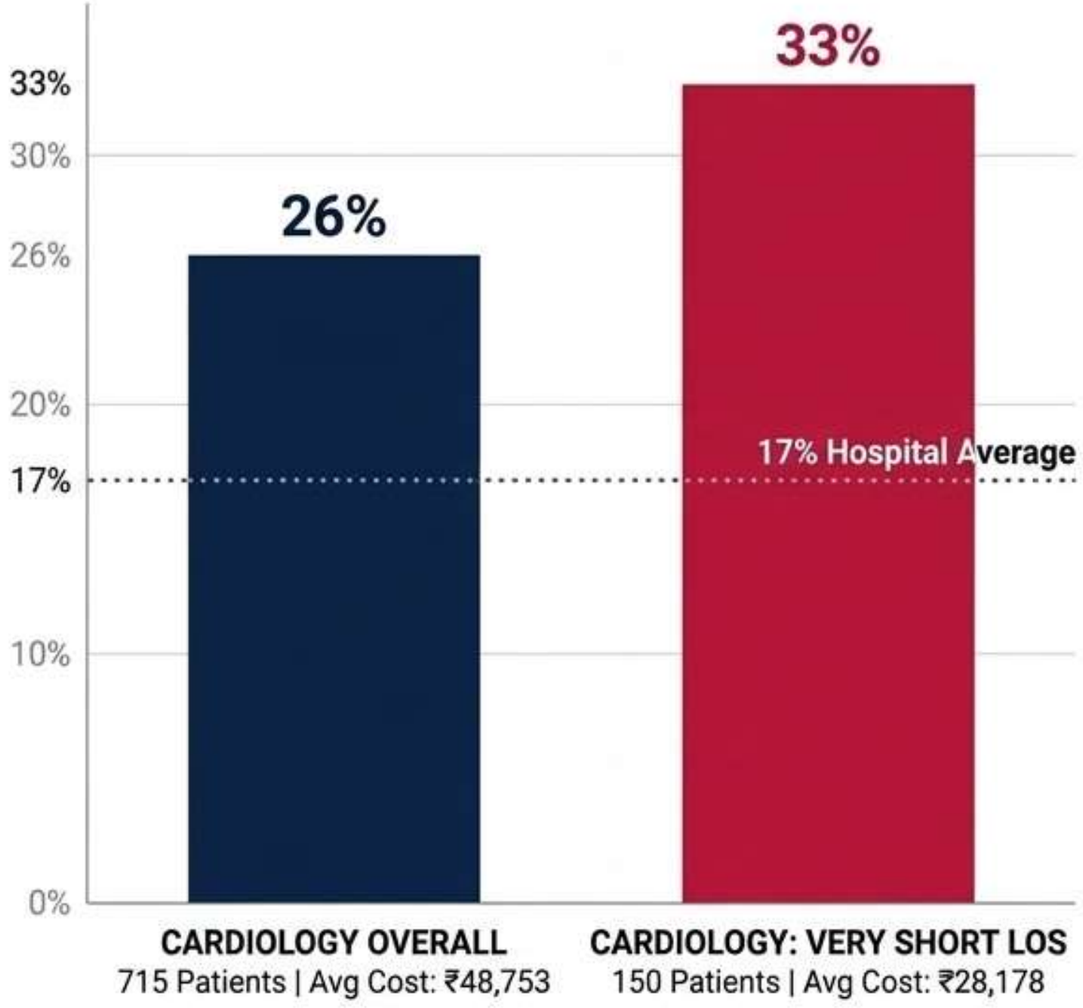
Risk Analysis: The 17% Readmission Problem

842 total patients required readmission. The overall 17% facility average is heavily skewed by severe clinical bounce-backs in Oncology and Cardiology.



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Synthesis Deep Dive: The Cardiology Anomaly



PREMATURE DISCHARGE FAILURE

Accelerating patient discharge to achieve a 'Very Short' LOS in Cardiology pushes the readmission rate to a critical 33%.

This negates any initial operational cost savings (₹28,178 artificial low cost) by generating high-risk, high-cost readmission cycles.
Action required: Extend LOS minimums for cardiac patients immediately.

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Strategic Business Insights & Directives

01

Standardize Cardiology Discharge

Extend minimum LOS thresholds for high-risk cardiac patients to mitigate the 33% readmission spike linked to premature discharges.

02

Audit Oncology Pathways

Investigate the severe operational disconnect between the system's highest resource intensity (₹48.97 Cr total) and its lowest patient satisfaction (3.60).

03

Dynamic Q4 Resource Allocation

Pre-scale staffing, ICU bed availability, and supply chains to capture maximum margin during the Nov/Dec (1,100+ patient) winter surge.

04

Payer-Mix Optimization

Prioritize billing and intake pathways for Private Insurance patients to maximize the system-high ₹7,809 per-day operational yield.

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